



Outpatient Clinical Care for Adults

APRIL 16, 2024

 For Health Care Providers

KEY POINTS

- Antibiotic prescribing guidelines create standards of care and improve quality of care.
- Over-the-counter medications can provide symptom relief but have not been shown to shorten the length of illness. There is limited evidence supporting specific symptomatic therapies.
- Healthcare professionals and patients should weigh the potential for benefits and adverse effects when considering symptomatic therapy.

Adult outpatient treatment recommendations

The table below outlines the most recent guidance for appropriate antibiotic prescribing for adults seeking care for common infections in an outpatient setting.

The table provides information related to over-the-counter medication for symptomatic therapy.

Condition	Epidemiology	Diagnosis	Management
Acute rhinosinusitis [1] , [2]	• About 1 out of 8 adults (12%) in 2012 reported receiving a diagnosis of rhinosinusitis in the previous 12 months, resulting in more than 30 million diagnoses. • 90–98% of rhinosinusitis cases are viral, and antibiotics may not help even if the causative agent is bacterial.	• Diagnose acute <u>bacterial</u> rhinosinusitis based on symptoms that are: ◦ Severe (>3-4 days), such as a fever ≥39°C (102°F) and purulent nasal discharge or facial pain; ◦ Persistent (>10 days) without improvement, such as nasal discharge or daytime cough; or ◦ Worsening (3-4 days) such as worsening or new onset fever, daytime cough, or nasal discharge after initial improvement of a viral upper respiratory infections (URI) lasting 5-6 days. • Sinus radiographs are not routinely recommended.	For a bacterial infection • Healthcare professionals should encourage watchful waiting for uncomplicated cases for which reliable follow-up is available. • Prescribe amoxicillin or amoxicillin/clavulanate, the first-line therapy. • Macrolides such as azithromycin are not recommended due to high levels of <i>Streptococcus pneumoniae</i> antimicrobial resistance (~40%). • For penicillin-allergic patients, prescribe doxycycline or a respiratory fluoroquinolone (levofloxacin or moxifloxacin) as alternative agents.

Condition	Epidemiology	Diagnosis	Management
Acute uncomplicated bronchitis [3] , [4] , [5]	Cough is the most common symptom for which adult patients visit their primary care provider, and acute bronchitis is the most common diagnosis in these patients.	- Evaluation should focus on ruling out pneumonia, which is rare among otherwise healthy adults in the absence of abnormal vital signs (heart rate ≥ 100 beats/min, respiratory rate ≥ 24 breaths/min, or oral temperature≥ 38 °C) and abnormal lung examination findings (focal consolidation, egophony, fremitus). - Colored sputum does not indicate bacterial infection. - For most cases, chest radiography is not indicated.	Routine treatment of uncomplicated acute bronchitis with antibiotics is not recommended, regardless of cough duration. Options for symptomatic therapy include: - Cough suppressants (codeine, dextromethorphan); - First-generation antihistamines (diphenhydramine); - Decongestants (phenylephrine). Evidence supporting specific symptomatic therapies is limited.
Common cold or non-specific upper respiratory tract infection (URI) [6] , [7]	- The common cold is the third most frequent diagnosis in office visits, and most adults experience two to four colds annually. - At least 200 viruses can cause the common cold.	Prominent cold symptoms include fever, cough, rhinorrhea, nasal congestion, postnasal drip, sore throat, headache, and myalgias.	- Decongestants (pseudoephedrine) combined with a first-generation antihistamine may provide short-term symptom relief of nasal symptoms and cough. - Healthcare professionals can give non-steroidal anti-inflammatory drugs to relieve symptoms. - Evidence is lacking to support antihistamines (as monotherapy), opioids, and intranasal corticosteroids as effective treatments for cold symptom relief. Healthcare professionals and patients must weigh the benefits and harms of symptomatic therapy.
Pharyngitis [8] , [9]	- Group A beta-hemolytic streptococcal (GAS) infection is the only common indication for antibiotic therapy for sore throat cases. - GAS causes only 5–10% of adult sore throat cases.	- Clinical features alone do not distinguish between GAS and viral pharyngitis; a rapid antigen detection test (RADT) is necessary to establish a GAS pharyngitis diagnosis - Those who meet two or more Centor criteria (e.g., fever, tonsillar exudates, tender cervical lymphadenopathy, absence of cough) should receive a RADT. Throat cultures are not routinely recommended for adults.	- Antibiotic treatment is NOT recommended for patients with negative RADT results. - Amoxicillin and penicillin remain first-line therapy due to their reliable antibiotic activity against GAS. - For penicillin-allergic patients, cephalexin, cefadroxil, clindamycin, or macrolides are recommended. - GAS antimicrobial resistance to azithromycin and clindamycin are increasingly common. - Recommended treatment course for all oral beta-lactams is 10 days. - For specific treatment recommendations and dosing, visit the Pharyngitis (Strep Throat) page for clinicians.
Acute uncomplicated cystitis [10] , [11]	Cystitis is among the most common infections in women and is usually caused by <i>E. coli</i>.	- Classic symptoms include dysuria, frequent voiding of small volumes, and urinary urgency. Hematuria and suprapubic discomfort are less common. - Nitrites and leukocyte esterase are the most accurate indicators of acute uncomplicated cystitis	For acute uncomplicated cystitis in healthy adult non-pregnant, premenopausal women: - Nitrofurantoin, trimethoprim/sulfamethoxazole (TMP-SMX, where local resistance is <20%), and fosfomycin are appropriate first-line agents. - Fluoroquinolones (e.g., ciprofloxacin) should be reserved for situations in

Condition	Epidemiology	Diagnosis	Management
			which other agents are not appropriate.

Adult Treatment Recommendations

SOURCES

CONTENT SOURCE:
[National Center for Emerging and Zoonotic Infectious Diseases \(NCEZID\)](#)

REFERENCES

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